Phesgo + vinorelbine

Phesgo on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by intravenous (IV) OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; any antibody-only phase is separately prescribed.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart / breathing

HER2 antibodies

Keep echoes; report breathlessness, swelling or chest symptoms.

2 Constipation / bowel pain

Vinorelbine

Report constipation early; severe pain, vomiting or inability to pass stool or wind needs urgent advice.

3 Blood / infection

Vinorelbine

Fever, shivers or sudden illness: contact oncology immediately.

4 Nerves

Vinorelbine

Report tingling, weakness or difficulty with usual tasks.

5 Reaction / injection site

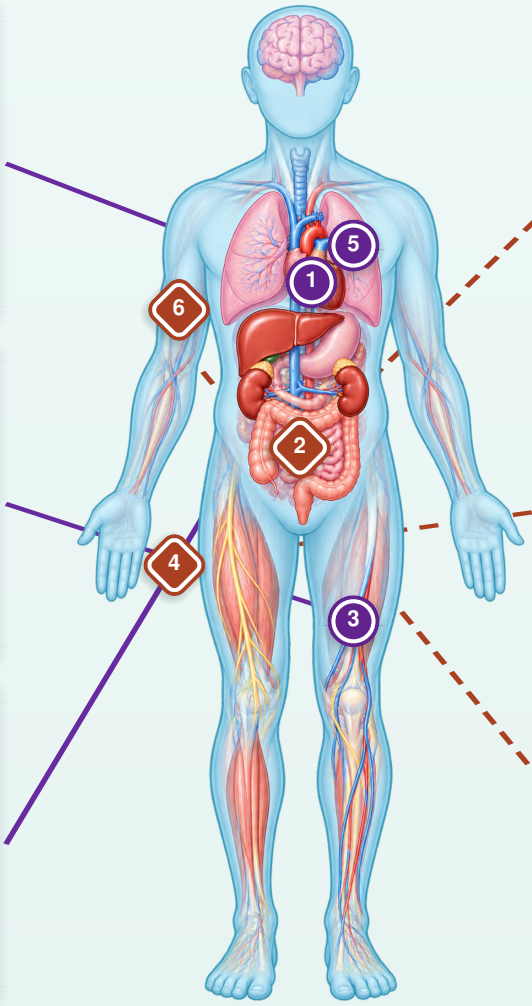
Phesgo

Tell staff immediately about rash, chest tightness or breathing difficulty.

6 IV site / oral nausea

Vinorelbine

IV burning or swelling needs immediate attention. Oral nausea needs a route-specific plan.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The map shows selected effects of the medicines named here. Report persistent effects from earlier treatment too.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Pertuzumab

- Heart pumping-function reduction
- Treatment reaction
- Diarrhoea and rash
- Fatigue

Trastuzumab

- Heart pumping-function reduction
- Treatment reaction
- Diarrhoea and rash
- Fatigue

Vinorelbine

- Low blood counts and infection
- Constipation or bowel obstruction
- Numbness or weakness
- Nausea; oral doses need a specific plan
- Irritation if an IV dose leaks

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Route-specific support

Oral vinorelbine needs a separate nausea plan; do not copy an IV-only minimal-risk template. Confirm anti-sickness treatment, constipation prevention and blood checks before each prescribed dose.

Tests before treatment

full blood count (FBC), kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose. Confirm the blood-count check needed before each additional vinorelbine or paclitaxel dose; a Day 1 result does not automatically authorise a later dose. For oral vinorelbine, the product information requires blood counts on the day of each administration, including Day 8; the regimen source lists full blood/renal/liver checks before each cycle.

Keep heart checks on the plan

An echo every 12 weeks is separate from blood tests or cancer imaging. Ask who books and reviews it.

Plan for symptoms

Keep your agreed contact numbers and instructions. New symptoms can need assessment before a routine appointment.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00798/

Sources and review status are available online. Follow your own prescription and hospital instructions.